NCCP 00105 · clinician draft

Ipilimumab alone — advanced melanoma

The treatment this guide covers This guide describes ipilimumab monotherapy at 3 mg/kg. It is not the nivolumab combination and does not automatically lead to nivolumab maintenance. A four-dose treatment course does not mean immune risks end after dose four.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Ipilimumab alone — advanced melanoma · support plan

Active medicines: Ipilimumab

Emetogenic risk: Low per NCCP 00105 v7

Medicine	Dose / route	When
Ondansetron	8 mg orally once daily	Day 1, if prescribed under the low-risk plan Current NCIS V8 Table 1 p4. Review QT, electrolytes, constipation, hepatic dosing and interactions; this reference requires an individual prescription.
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days; duration to be prescribed Review contraindications, renal function, interactions and extrapyramidal adverse effects.

Additional phase-specific support

- The medicine list is a source-based low-risk reference for an individual consultant-approved/local prescription; it is separate from infusion-reaction premedication and immune-toxicity steroid treatment.
- NCCP classifies ipilimumab as low emetogenic risk; anti-sickness treatment is individual. Routine infusion premedication is not usually required, but previous reactions may change the plan. Do not start steroids or alter prescribed steroid tapers or hormone replacement without the team.
- Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, plus a careful symptom review. Cortisol and other tests are added when symptoms suggest endocrine or other immune effects. Normal routine blood results do not exclude colitis, lung, nerve or other inflammation.
- Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.
- Discuss pregnancy prevention, fertility and breastfeeding before treatment. Effective contraception and the duration after the last dose must be confirmed against the current ipilimumab product information and your written plan; this guide does not borrow the nivolumab five-month interval. Tell the team promptly if pregnancy is possible.
- This guide describes ipilimumab monotherapy at 3 mg/kg. It is not the nivolumab combination and does not automatically lead to nivolumab maintenance. A four-dose treatment course does not mean immune risks end after dose four.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00105 v7 · p4 · NCCP NCIS medical-oncology antiemetics V8, Table 1 p4, low risk

Sources checked 2026-11-06 · SACTCheck v0.84.42 · Regimen-level drafts; prescriber and oncology-pharmacy verification required